Royal Devon University Healthcare NHS Foundation Trust

End of Life Care (Adults) Policy Trustwide	
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Note: This document has been assessed for any equality, diversity or human rights implications

Controlled document
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Version	Date	Author	Reason
1.0	01/09/2015	End of Life Education Facilitator and Clinical Leads	Updated policy, to embed the principles of One Chance to Get it Right and incorporate NICE guidelines for Care of the Dying Adult.
2.0	07/12/2016		Intranet links (only) updated.
3.0	18/06/2021	Joint End of Life Lead	Review of policy in light of Strategy, Quality Objective and updated National Ambitions paper,
4.0	28/11/2023	EOL Leads (Eastern & Northern) and Palliative Care Nurse Consultant (Eastern)	Review of policy in light of integration of Eastern and Northern services following Trust merger on 01.04.2022,
4.1	31/07/25	EOL Leads (Eastern & Northern) and Palliative Care Nurse Consultant (Eastern)	Addition of governance structure at Appendix 4.

Associated Trust Policies / Procedural documents:	End of Life Strategy [add link when live] Cardiopulmonary Resuscitation (CPR) Policy (Eastern) / Resuscitation Policy (Northern) Care of the Deceased Adult Patient Policy (Eastern) / Care of the Deceased SOP (Northern) Donation of tissue for Transplantation Policy (Eastern) / Tissue Donation Policy (Northern) Mental Capacity Act (MCA) Policy (Trustwide) Information Governance Policy (Trustwide) Privacy, Dignity, Chaperone and Compassion Policy (Eastern) / Dignity & Privacy Policy (Northern) Spiritual Care Policy (Eastern) Advance Decision to Refuse Treatment Policy (Eastern) / Deactivation of Implantable Cardioverter Defibrillators (ICD) towards the end of life Just in Case Bags for use in End of Life Care Advance Care Planning Policy
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KEY POINTS OF THIS POLICY:

- Identification of patients requiring End of Life (EOL) care, regardless of diagnosis. This will include patients for whom recovery is uncertain, patients possibly in the last 12 months of life and patients in the last few days/hours of life.
- Provides guidance on holding, documenting and sharing advance care planning discussions including Treatment Escalation Plan (TEP) discussions.
- It defines the requirements, standards and actions necessary to deliver high-quality care in the last few days of life across the organisation.
- Outlines the requirements and actions necessary to facilitate, whenever possible, a rapid discharge for those patients in the last few days of life who wish to die at home, or elsewhere, through collaborative and partnership working with other agencies.

1. INTRODUCTION

1.1 Caring for people at the end of life and providing support to their families and friends at this time is of profound importance. There is only one chance to get it right for the individual patient and the impact of that death will live on in the memories of those close to them.

1.2 This policy outlines the responsibilities of the organisation and provides guidance to staff on achieving and maintaining high standards of end of life care for adult patients and their relatives and friends. It has been developed to ensure that all staff working for/within The Royal Devon University Healthcare NHS Foundation Trust (hereafter referred to as the Trust) can deliver high quality and compassionate EOL care. This includes patient and staff in all settings: acute hospital and community.

1.3 This includes the support and care given to relatives, friends and carers. For those in the last days of life the '5 Priorities of Care' (One Chance to Get it Right by the Leadership Alliance for the Care of Dying People, June 2014) will be met. The philosophy of holistic care should also continue after death and into bereavement with support being available for family, friends and carers as appropriate.

It will ensure that those patients, who are dying, regardless of care setting, will receive the best possible care. Where patients are in hospital we will work with the patient and their family to support discharge to their preferred setting. This will also include working across the health care system to meet their needs.

1.4 The provision of EOL care is not confined to specialist palliative care services. The Trust recognises that EOL care is a vital and integral component of health and social care: it is everyone's business. The policy applies to all staff in both clinical and support roles who have contact with patients in their last year of life. This includes temporary, locum, bank, agency and contracted staff.

1.5 The Trust supports John's Campaign (www.johnscampaign.org.uk) This is a simple campaign that aims for the right to stay with people with dementia in hospital. The campaign involves working with families and carers as 'partners in care' to ensure open visiting and access to the person in hospital at the end of their lives.

1.6 All staff working within the Trust and visiting professionals have a responsibility to ensure that all patients have their end of life care needs met. This includes the needs of relatives and friends during and after the death of the patient has occurred.

1.7 EOL care is supported by the voluntary and charity sector across Devon, these organisations (e.g. Local Hospices and Marie Curie, hospital charities, etc) provide a critical role in the care of the EOL patients and their loved ones.

1.8 EOL care is one of the eight core services inspected by CQC in acute hospitals. As a result, the quality of end of life care provided at all levels, from the patient's bedside, through to strategic management, has potential to influence the Trust's inspection outcomes. Hence all staff have a part to play in striving to deliver outstanding End of Life care.

1.9 The Trust recognises the need to support standards and a programme of education for all staff to deliver high quality, compassionate care to patients who have an advanced, life-limiting illness and their family and carers.

1.10 This guidance is underpinned by the National End of Life Care Strategy and the Trust End of Life Strategy which encompasses The Ambitions for Palliative and End of Life Care: A national framework for local action, 2021-26 In order to provide good end of life care for all patients it emphasises the following:

- Identification of people approaching the end of life.
- The opportunity to discuss personal needs and preferences
- Co-ordination of care and support
- Rapid specialist advice and clinical assessment
- Delivery of high-quality care and support in the last days of life
- Services that treat people with compassion dignity and respect
- Appropriate support and advice for carers
- Care after death.

1.11 EOL care is currently a Quality Objective for the Trust and this forms part of our overall plan to ensure the Trust EOL strategy is successfully implemented and embedded; see Appendix 3.

1.12 These principles are supplemented by Guidance and Recommendations from More Care, Less Pathway, Once Chance to get it Right and NICE Guidelines for the Care of the Dying Adult and NHS England – NHS Chaplaincy Guidelines 2015, Promoting Excellence in Pastoral, Spiritual & Religious Care and Review of Liverpool Care Pathway for dying patients - GOV.UK.

1.13 This policy is to be used in the care of all patients who are identified as being within the last year of life, with specific application of the five priorities of care to those patients who are within the last few days of life. Those patients for whom death is unexpected should be managed under the Resuscitation Policy (Cardiopulmonary Resuscitation (CPR) Policy (Eastern) / Resuscitation Policy (Northern)) although the principles of this policy are applicable to their care in the "peri-arrest" period and after death.

2. PURPOSE

2.1 To enable all Trust staff to provide high-quality person-centred care to people who are felt to be in their last year of life.

2.2 To support staff confidence, communication and partnership working, through provision of standards of practice and education.

2.3 To ensure that high-quality and compassionate care is delivered in a way that respects the privacy, dignity, wishes and needs of all patients at the end of life.

2.4 To enable patients to express their preferred place of dying and for this to be facilitated as far as is reasonably possible.

2.5 To ensure that the carers are supported, appropriately informed, enabled and empowered throughout their EOL care and in the bereavement period of their relative or friend.

2.6 To ensure there is a robust review and targeted education where deficiencies of care are identified.

3. DEFINITIONS

3.1 **End of Life Care** - End of Life (EOL) care has been defined as patients who are 'approaching the end of life' when they are likely to die within the next 12 months. This includes patients whose death is imminent (expected within a few hours or days) and those with:

- Advanced, progressive, incurable conditions
- General frailty and co-existing conditions that mean they are expected to die within 12 months
- Existing conditions if they are at risk of dying from a sudden acute crisis in their condition
- Life-threatening acute conditions caused by sudden catastrophic events.

Source: Treatment and care towards the end of life - professional standards - GMC

3.2 Within the National EOL strategy, End of Life care is defined as that needed in the last year of life. For many long-term conditions this represents the end-stage of an illness. The exception to this is for people living with dementia, for whom the end- stage of illness may be protracted for longer than a year. Source: National Council for Palliative Care 2006, The End of Life Strategy 2008.

3.3 **Palliative Care -** The World Health Organization (WHO) defines palliative care as an approach that improves the quality of life of patients and their families facing the problems association with life-threatening illness, through the prevention and relief of suffering by means of early identification and impeccable assessment and treatment of pain and other problems, physical, psychosocial and spiritual.

3.4 **'Family**' refers to carers, friends, relatives - anyone the patient regards as family.

3.5 **Advance Care Planning (ACP**) - is the process of discussing and planning ahead in anticipation of no recovery from or a deterioration in a patient's condition.

It should also include discussions around the ceiling of care and the communication to the wider MDT on joint decisions made. This is important for ensuring patient-focussed care and is central to end of life care. The process of discussing this can be seen as part of the solution in that it enables emotional 'catch up' and adaptation to the new reality and normalisation of life. Sensitive discussion of advance care planning can strengthen coping mechanisms and enable realistic planning. There is some evidence that it increases rather than decreases realistic hope. It also enables health and social care services to anticipate need and through appropriate and timely direction of resource avoid crises and use of emergency services. NHS England » Universal Principles for Advance Care Planning (ACP)

3.6 An **Advance Decision to Refuse Treatment** (ADRT) is a statement made by a mentally competent person aged over 18 years, which defines in advance their refusal of medical treatment should they become mentally or physically incapable of making their wishes known. To be legally binding the ADRT should be appropriately signed and witnessed, should be applicable to the condition being considered and should include a statement indicating that the patient understands that their life may be at risk if treatment is refused.If a patient has an LPOA for Health and Well-being the ADRT is null and void.

3.7 **Advance Statements** may be made to express patient's wishes, feelings, beliefs and values about future care. Unlike the Advance Decision, an Advance Statement is not legally binding but must be considered if the person loses capacity as it can add to the clinical team's ability to make a "best interests" decision about that person's care. Advance Statements may be made verbally or in writing.

3.8 **Lasting powers of Attorney** (LPAs) were created under the Mental Capacity Act 2005, and came into effect on 1 October 2007. Their purpose is to meet the needs of those who can see a time ahead when they will not be able – in the words of the Act, will lack capacity – to look after their own personal and financial affairs.. The LPA allows them to make appropriate arrangements for family members or trusted friends to be authorised to make decisions on their behalf. The LPA document must be sighted to ensure and understand any caveats attached to this and to establish the type of LPA There are two types of LPA:

- Health and welfare (includes medical care)
- Property and financial affairs.

3.9 **"Best interests" decisions.** A "best interests" decision must be made by the treating medical team if a patient has lost mental capacity and there is no appointed LPA for Health and Welfare. LPAs for health and welfare legally allow the holder of the LPA to speak on behalf of the patient to establish goals of care and treatment. The aim of this decision-making process is to act in the patient's best interests if they are no longer able to give valid consent of their care and treatment. ADRTs allow the patient to give their legally binding view if they lose capacity (but are superseded by the appointment of an LPA). Best interests' decisions by medical staff must take previously expressed wishes, ADTRs and advance statements into consideration when formulating "best interests" plans for care and treatment. The treating team must also speak to the patient's family (as defined in 3.4 to gather information about the patient's more informal wishes. It is best practice to include family and the patient where possible (even if lost capacity) in the process of "best interests" decision making if the patient themself has lost capacity. This ensures that the patient's values and wishes are taken

into consideration by the clinical team and also, importantly, gives the family information and maintains their involvement in planning care and treatment. Good communication with families at the EOL is likely to reduce family distress and is likely to improve their bereavement.

3.10 **The Five Priorities for Care of the Dying Person.**

In June 2014, the government's Leadership Alliance for the Care of Dying People published its report called: 'One change to get it right: Improving people's experience of care in the last few days and hours of life. This described five 'Priorities for Care of the Dying Person,' based on the needs and wishes of the dying person and those close to them. The new approach recognises that in many cases, enabling the individual to plan for death should start well before a person reaches the end of their life.

The Priorities for Care are set out as:

- The possibility that a person may die within the coming days and hours is recognised and communicated clearly, decisions about care are made in accordance with the person's needs and wishes, and these are reviewed and revised regularly by doctors and nurses.
- Sensitive communication takes place between staff and the person who is dying and those important to them.
- The dying person, and those identified as important to them, are involved in decisions about treatment and care.
- The people important to the dying person are listened to and their needs are respected.
- Care is tailored to the individual and delivered with compassion – with an individual care plan in place.

The aim is to promote a stronger culture of compassion in the NHS and social care – one that puts people and their families at the centre of decisions about their treatment and care.

3.11 Electronic Palliative Care Co-ordination System (EPaCCS) is a term applied to registers of patients identified as being in receipt of EOL care that can be accessed by OOH services, emergency responders, etc. In Devon, this online register is co-ordinated by the OOH GP service (currently Practice Plus Group). It enables Community Teams, GP's and hospice organisations to access information with regards to patients' preferences and what is available in terms of support and medications at home.

3.12 The Trust's Electronic Patient Record (EPR), Epic, includes the identification of dying, linking to the 5 priorities of care this will facilitate the framework to capture the relevant key performance indicators (KPI's) to monitor the quality of EOL care.

3.13 **Spirituality** – Among others, two definitions are as follows:

- 'Spirituality is the dynamic dimension of human life that relates to the way persons (individual and community) experience, express and/or seek meaning, purpose and transcendence, and the way they connect to the moment, to self, to others, to nature, to the significant and/or the sacred.'
  European Association of Palliative Care 2010: [https://www.eapcnet.eu/eapc-groups/reference/spiritual-care/]

- ‘Spirituality is defined as 'those beliefs, values and practices that relate to the human search for meaning in life. For some people, spirituality is expressed through adherence to an organized religion, while for others it may relate to their personal identities, relationships with others, secular ethical values or humanist philosophies.’

4. DUTIES AND RESPONSIBILITIES OF STAFF

4.1 Executive Leadership

4.1.1 The **Chief Nursing Officer** is the Lead Executive Director for End of Life Care. Strategic leadership is, however, provided jointly with the Chief Medical Officer. The Chief Nursing Officer has overall responsibility for ensuring processes are in place to provide patients with appropriate End of Life care and for ensuring that this policy is reviewed and that there are appropriate quality assurance mechanisms in place in relation to the guidance in this policy.

4.1.2 The **Chief Medical Officer** will provide clinical and cultural leadership which recognizes that care of a dying person is a priority and a responsibility of all healthcare staff.

4.2 Care Group oversight

4.2.1 The **Care Group Directors** hold accountability for patient safety within their sites, including safety relating to this strategy and the care of EOL patients within their services.

4.2.2 The **Care Group Directors for Patient Care** are responsible for ensuring that the processes outlined in this strategy are implemented within their Care Groups.

4.3 All Healthcare Professionals

4.3.1 **Lead clinicians (medical and non-medical**) should signpost clinical teams and members to read the policy and understand their role, actions and responsibilities they have for all end of life patients. They will champion this policy in practice. They will signpost their teams to the relevant documentation and patient information leaflets to support EOL care. These are available on the Specialist Palliative Care Teams’ intranet pages.

4.3.2 **Medical and registered nursing staff** are responsible for adhering to the policy which provides a framework for best practice; contributing to multidisciplinary decisions; completing and coordinating individualized care plans; supporting advance care planning discussions, completing all relevant documentation to evidence care delivery.

4.3.3 **Allied Health Professionals** will contribute to multidisciplinary decisions, advance care planning discussions and individualized care plans and will support patients to achieve their goals at end of life.

4.3.4 **Any member of staff** who has concerns about any aspect of a patient’s care towards the end of life should escalate it immediately to a more senior member of the multidisciplinary team and document the concern via Datix.

4.3.5 All staff must operate within the legal frameworks provided by the Mental Capacity Act 2005 and its code of practice; and their regulatory body.

4.4 Last Few Days of Life

4.1.1 The **senior responsible clinician** will be responsible for supporting the MDT to identify patients thought to be in the last few days of life and for planning, reviewing, coordinating, communicating and delivering an individualized care plan (ICP) in conjunction with the multi-disciplinary team and in line with the five priorities of care.

4.1.2 **All Health Care Professionals** – any professional with direct responsibility for a patient or for an aspect of the patient's care and treatment must respect the five priorities of care of the dying. They must ensure that they maintain the skills and competencies appropriate to their role to manage the assessment, communication and symptom management of patients who are in the last few days of life. They must raise any need for additional training with their line manager and review their experience and competencies within their PDR/Appraisal Review.

4.5 Clinical Governance

4.5.1 The End of Life Steering Group

The EOL Steering Group has been established to:

- Develop a EOL Strategy for the Trust.
- Oversee the implementation of the Trust EOL Strategy and supporting action plan.
- Provide oversight and support of the Care Quality Commission (CQC) compliance in relation to EOL across all Care Groups.
- Provide oversight and assurance of the quality standards being delivered for EOL care (EOLC) for the Trust over time.

4.5.2 The EOL Steering Group reports into and is accountable to the Patient Safety Committee (PSC) of the Trust. Ratification of guidelines/documents will be reviewed by the Clinical Effectiveness Committee (CEC).

5. PROCEDURE

5.1 Planning and Delivering End of Life Care

End of Life care planning must be a multi-agency process which fully involves the patient and/or relatives and friends at all stages. Patients may access Trust services at various stages of their illness and receive care, treatment and support from a range of services. Meticulous attention to communication within and across care settings and coordination of care is vital.

5.2 Recognising Patients who need End of Life Care

5.2.1 This can be done by any healthcare professional confident and competent in recognising that a patient may be in the final year of their life and/or identifying that a patient is dying. This may be an MDT led decision. Identified

patients should be informed and offered opportunities to have/record advance care planning discussions.

5.2.2 Recognising patients for whom recovery is uncertain:

- Is the patient deteriorating, clinically unstable with limited reversibility? And
- Is the patient at risk of dying during this episode of care despite treatment?

5.2.3 An MDT decision can be made to instigate parallel planning ('hoping for the best whilst planning for the worst') and to support advance care planning discussions to establish a patient and their family's priorities and preferences for care.

5.3 Holistic Needs Assessment

5.3.1 All patients thought to be in the last year of life should have a holistic needs assessment at each key point of their care. Assessment of physical, psychological, social and spiritual needs should be undertaken and documented.

5.3.2 Once a holistic assessment has been carried out, it is important that this is shared appropriately in order to co-ordinate care for the patient, families and friends, and carers. Trust staff will need to work closely with the healthcare community and social services

5.4 Spiritual Needs

5.4.1 Patients' spiritual needs should be assessed and may need referral to other agencies. Referral to spiritual caregivers, such as the Department of Chaplaincy and Spiritual Care, should be undertaken as appropriate.

5.4.2 This department's webpage on the Trust intranet also contains information about different religions and faiths, to assist staff in resourcing appropriate religious care.

5.5 Treatment Escalation Plan/ Resuscitation Decision Record, TEP form (Details in policy links and Cardiopulmonary Resuscitation (CPR) Policy (Eastern) / Resuscitation Policy (Northern))

5.5.1 When patients are thought to be in the last 12 months of life then TEP discussions should be considered. A sensitive explanation about escalation of treatment is likely to involve decisions about hospital admission, artificial nutrition and hydration, ventilation and CPR.

5.5.2 It is imperative that the patient is asked whether they would like to be involved in discussing their treatments, including CPR. If they want to be involved in these discussions then healthcare professionals are obliged to involve them.

5.5.3 TEP discussions should take place as early as possible and preferably when the patient is well enough to be involved. Discussions can take place in outpatients, in the community or indeed be facilitated across settings such that the hospital and primary care teams can support realistic discussions

mindful of the limitations of care available in their respective settings. Treatment decisions in hospital should involve senior clinicians.

5.6 Discharge Planning and Rapid Discharge for End of Life Care

5.6.1 For planning discharge from hospital, there needs to be a comprehensive coordinated discharge plan which should be ward led and/or supported by the Hospital Discharge Team. Teams should also contact Community Nursing to discuss the provision of support available and if the patient is known to the service. This will include assessment of patient and family requirements regarding Preferred Place of Care (PPC) and link in with Health and Social Care professionals to support the patient's end of life wishes.

5.6.2 Fast-track application for Continuing Health Care (CHC) funding should be initiated for any patient requiring an urgent package of care, who is rapidly deteriorating, likely to be entering the terminal phase of their illness, and is expected to die. Fast Track funding gives non-means tested funded care packages/nursing home placement to support the patient in their preferred place of care. This will be undertaken when the patient is discharged from hospital to their place of care.

a) If the patient is at home or in a residential care home, then assessment may be undertaken by the Community Nursing Team, in collaboration with community allied health professionals, GP, and the community specialist palliative care team (where already involved with the patient).

b) The fast-track CHC assessment enables the patient's end of life needs to be clearly identified and an adequate package of care to be sourced in order to support EOL care in the community setting, thereby avoiding crises at home, and admissions to hospital due to lack of appropriate care/support.

5.6.3 All discharging patients thought to be at the end of life should be referred to community nursing services, with details of their needs discussed and communicated to the community team. These should also be communicated to the GP via the discharge letter from the clinical team or via a telephone call if the patient is thought to be in the last days of life.

5.6.4 If a patient has been identified as requiring rapid discharge in the last few days of life please refer the patient to Hospital Discharge Team to facilitate a safe discharge.

5.7 Advance Care Planning

5.7.1 An advance care plan is the result of a discussion between the patient (and those close to them if the patient wishes) and healthcare professional that addresses a patient's wishes for their future care. It is a key means of improving care for people whose recovery is uncertain or who are nearing the end of life, enabling better planning and provision of care, to help them live and die in the place and the manner of their choosing. It is a dynamic process which evolves over time.

5.7.2 Advance Care Planning can ensure that relatives and friends understand what the patient's preferences for care are at a time when they cannot be communicated.

5.7.3 Where the patient does not have the mental capacity to participate in these discussions, families should be involved. For patients without relatives and friends the IMCA service should be used.

5.7.4 The pertinent outcomes of Advance Care Planning discussions, Advance Statements, priorities, preferences or wishes in an inpatient setting should be:

- Recorded in the patient's electronic record;
- Include Treatment Escalation Plans and Resuscitation Decision Records (TEP/RDR);
- Communicated to the GP;
- Included in the Discharge summary;
- Professionals who are able to access the Electronic Palliative Care Co-ordination (EPaCCs) register, this information should be recorded here where appropriate.

5.7.5 For those patients who are in a community setting, The GP and the MDT will be jointly responsible for care prescription and completion of documents. This will include documentation of the ceiling of care agreed by patient and family and recorded on the TEP/RDR form.

All discussions and documentation relating to TEP/RDR should be carried out according to the Trust **Cardiopulmonary Resuscitation (CPR) Policy (Eastern)** / **Resuscitation Policy (Northern)**.

5.8 Deactivation of Implantable Cardioverter Defibrillators (ICDs)

5.8.1 Patients with ICDs may develop terminal illness due to worsening of their underlying heart disease or other chronic non-cardiac disease. Patients at the end of life are more likely to develop conditions such as hypoxia, sepsis, pain, heart failure and electrolyte disturbances, predisposing them to arrhythmias and thus increasing the risk of shock therapy. Shocks can be physically painful and psychologically stressful, without prolonging a life of acceptable quality, a result of which is inconsistent with comfort care goals.

Furthermore, near the end of life, patients may either not wish to undergo cardiopulmonary resuscitation (CPR) or CPR may not be medically appropriate, therefore ICD tachycardia therapy deactivation should be considered when the patient's clinical status worsens and death is near.

5.8.2 Criteria for deactivating a defibrillator should be discussed with a patient and/or their family. Ideally, discussions should take place while the patient is still able to be involved in the decision-making process, including the ability to sign a consent form for ICD deactivation.

5.8.3 If the patient does not have mental capacity to participate in the discussion then the decision should be guided by an ADRT or LPA if present. In the absence of such documents, a best interests decision should be made by the clinical team with the involvement of the family/IMCA.

5.8.4 Deactivation should be performed under the Trust Wide policy 'Deactivation of Implantable Cardioverter Defibrillators (ICD) towards the end of life.

https://royaldevonstaff.nhs.uk/dowload.cfm?ver=3411

https://royaldevonstaff.nhs.uk/download.cfm?ver=25199

5.8.5 If urgent deactivation of an ICD is required outside of standard working hours, then this can be done by placing a large magnet over the ICD. Such magnets can be obtained in an emergency from **CCU extension 2837/01392 402837 (Eastern) or Cardiology Pacing Unit 2475 / 01271 322475 (Northern).**

5.9 Care of the Dying Patient (in the last few days or hours of life)

5.9.1 If the possibility that a person may die within a few days/hours is recognised, and it is felt that it is no longer appropriate to pursue correction/treatment of potentially reversible factors, then a senior clinician in consultation with the multidisciplinary team should document this situation.

5.9.2 For patients recognised as being in this dying phase, teams should add the flag called 'End of Life' to the patient's EPIC chart, and should commence completion of the 'Last Hrs Days of Life' flowsheet (which is listed as a tab under Ward Navigator). In that flowsheet, clicking on each Priority heading displays a full drop-down list of questions to be completed in consultation with the patient and their family. Completion of this flowsheet helps to ensure the 5 'Priorities for Care of the Dying Person' are addressed.

5.9.3 It is important that where possible a full discussion is undertaken with the patient and family. It should be explained that the patient is thought to be in the dying phase, and the reasons for this opinion. Discussions should include:

- the likely course of their condition
- issues such as nutrition and hydration
- cardiopulmonary resuscitation and treatment escalation plan
- decisions to stop routine recording of vital signs ('observations'), and glucose monitoring in diabetic patients
- symptom management
- the need for psychological, spiritual and social support
- preferred place of death
- Family communication preferences and contact details

Such discussions should be recorded in notes.

5.9.4 Once a senior clinician has concluded the patient is in the dying phase, an 'End of Life' flag should be added to the patient's EPIC chart.

5.9.5 Families should be offered the patient information leaflet regarding caring for someone in the last days of life and other relevant information they may find useful. In particular it should be discussed how the family may contribute to the patient's care should they wish. Leaflets available include:

- Treatment Escalation Plan (TEP) and Cardiopulmonary Resuscitation (CPR)
- End of Life - Coping with dying
- End of Life Care - Why won't they eat
- End of Life Care - Fluids and artificial hydration
- Information for the family and carers of those in the last few days of life

5.9.6 The patient should be reviewed at least daily by a senior doctor and senior nurse to assess for potential signs of recovery / comfort and to review the overall care plan. For community teams, patients should be reviewed by a senior nurse or GP where able.

5.9.7 It is important that a patient's symptoms are actively managed in accordance with their wishes, using the minimum (but adequate) doses of medication required to provide good control of symptoms. Where appropriate, non-pharmacological interventions to relieve symptoms should be used. Distressing symptoms maybe anticipated at the end of life, so appropriate PRN medication should be prescribed. This should be prescribed on appropriate drug charts depending on the clinical setting. For a patient being discharged home, anticipatory medications in the form of JIC bag will be helpful in avoiding any delay in symptom control. Prescribing Guidelines for relief of common symptoms experienced at end of life are available on the Trust Intranet Site, on each ward and the Joint Formulary for our community workforce. If symptoms fail to be rapidly controlled this should be escalated to a senior clinician for review and if necessary further to the Specialist Palliative Care Team both in the acute and community setting. https://northeast.devonformularyguidance.nhs.uk/formulary/chapters/16-palliative-care

5.10 Specialist and Palliative Care Team

5.10.1 There is access to specialist palliative care advice, through local Specialist Palliative Care Teams 7 days a week.

5.10.2 **Eastern Services -** Monday-Friday 9am-17:00pm, Saturday and Sunday 10:00 - 15:00. Outside of these hours the Hospice Team can be contacted on Ext 2586, there is an on-call Palliative Care doctor who can give telephone advice at all times. This on call doctor can give telephone advice and may also visit patients on a weekend on the ward if needed between the hours of 0900 - 1500.

5.10.3 **Northern Services -** 08:30 – 17:00 Monday to Friday. Outside of these hours colleagues at North Devon Hospice provide a telephone advice line on 01271 344248 or 01271 347 214. The Hospice has an on-call doctor who can give telephone advice at all times.

5.11 Care after death

5.11.1 After the patient's death the Trust Care of the Deceased Adult Patient Policy should be used to ensure continuation of high-quality care and provide appropriate support and information to relatives and friends. All beliefs of the patient or family must be respected in this process.

5.11.2 Where patients are registered on the Organ Donation Register and /or relatives express an interest in donating tissue, consent should be obtained in accordance with the Trust Donation of tissue for Transplantation Policy.

5.11.3 The medical team/practitioner should ensure that the 'Medical Certificate of the Cause of Death' are completed in a timely fashion, in collaboration with the Medical Examiners.

5.11.4 For inpatient deaths the ward team should complete a discharge summary for each patient providing detail of the circumstances surrounding the death as well as cause of death.

5.11.5 The family should be signposted to a member of the medical team if they wish to ask further questions regarding their relative's care, at any time in the future.

5.11.6 The paragraphs above focus mainly on deaths that occur in hospital. Care after expected deaths in community settings (e.g. at home or in care homes) should be managed according to current primary care practices and guidance.

5.12 Complaints and Concerns

5.12.1 All incidents and/or concerns regarding a patient's EOL care should be recorded on Datix, whether expressed by a member of staff, patient or relative.

They will be investigated by the relevant Care Group, however all such incidents are reviewed by the Lead Nurse for End of Life. They are reviewed thematically in clinical governance meetings of the Community Care Group, e.g. specialty governance meetings of the Specialist Palliative and EOL Teams. A dashboard has been developed within the Trust's risk management system (Datix) to display all incidents and complaints flagged as being related to EOL care. This data will be presented and monitored through the Trust's EOL Steering Group. Learning from incidents and complaints is fed back via the end of life link nurses and ward matrons.

5.13 Education and Training

5.13.1 The Specialist Palliative Care Teams across Devon will work with both internal Trust colleagues (e.g. Clinical Training Team, Resuscitation Team, and Care Homes Nurse Educator); and external stakeholders and partners (e.g. North Devon Hospice and Hospiscare) to ensure that all staff groups have access to appropriate training in End of Life Care.

5.13.2 Education and training will be delivered using a range of different formats, including the following:

- Case-based learning, e.g. at Care Group Clinical Governance or Mortality & Morbidity Meetings
- Newsletter on induction for all New Staff
- Online training modules for staff-role dependent
- Link Nurse programme
- Monthly lunchtime drop in sessions

- Opening Spiritual Gate course- delivered 4-6 times a year
- Support the Advanced Communication Skills Training
- Support for Syringe Driver Training and JIC medications
- Bite-size sessions delivered on wards by the team
- Education sessions delivered to medical staff as part of departmental or postgraduate training programmes; and bespoke sessions

5.13.3 Commitment to be active supporters of National Incentives to support conversations around death and dying i.e. Dying Matters week.

5.13.4 Learning from key national guidance and audits regarding EOLC i.e. National Audit for Care at the End of Life.

6. ARCHIVING ARRANGEMENTS

The original of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet Hub. Archived electronic copies will be stored on the Trust's “archived policies” shared drive, and will be held indefinitely.

7. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY

7.1 To evidence compliance with this policy, the following elements will be monitored:

	What areas need to be monitored?	How will this be evidenced?
1.	Priorities of Care to be used throughout the Trust for patients thought likely to be dying. All patients in last few days of life to be reviewed daily by senior clinician. Recognition of the Dying Phase.	Monthly last 72 hours of life Audit and National Audit for Care at the End of Life (NACEL) Annual Report
2.	EPaCCS- details of ACP undertaken should be recorded on EPaCCS	Local audits by appropriate teams in collaboration with stakeholders of integrated care systems.
3.	Participation in National Audit of Care at the End of Life (NACEL) by NHS Benchmarking Network	National publication and presentation of results within Trust
4.	Education and training - Provision to appropriate staff groups	Feedback from attendees. Annual Report
5.	Service user experiences of EOL care will be monitored through • surveys of bereaved relatives and/or focus groups • feedback portals about patient experience, e.g. Care Opinion	Surveys of bereaved relatives Minutes of Specialty Governance Group for Specialist Palliative and End of Life Care

	• complaints and compliments received • incidents reported	Minutes of End of Life Steering Group

8. REFERENCES

Advance Decision to Refuse Treatment (ADRT)
https://www.nhs.uk/conditions/end-of-life-care/advance-decision-to-refuse-treatment/

Ambitions for Palliative and End of Life Care: National Framework for Local Action (2021 - 2026)
https://acpopc.csp.org.uk/documents/ambitions-palliative-end-life-care-national-framework-local-action-2021-2026

Care Quality Commission
http://www.cqc.org.uk

End of Life Strategy 2008
https://www.gov.uk/government/publications/end-of-life-care-strategy-promoting-high-quality-care-for-adults-at-the-end-of-their-life

Gold Standards Framework
https://goldstandardsframework.org.uk/

More Care, Less Pathway- A review of the Liverpool Care Pathway.
More Care, Less Pathway

Mental Capacity Act

https://www.legislation.gov.uk/ukpga/2005/9/contents

https://www.scie.org.uk/mca/introduction/mental-capacity-act-2005-at-a-glance/

NICE Guidelines for the Care of the Dying Adult
https://www.nice.org.uk/guidance/ng31

NHS England – NHS Chaplaincy Guidelines 2015, Promoting Excellence in Pastoral, Spiritual & Religious Care
https://www.england.nhs.uk/wp-content/uploads/2015/03/nhs-chaplaincy-guidelines-2015.pdf

One Chance to get it Right
https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/323188/One_chance_to_get_it_right.pdf

Preferred Priorities of Care
https://www.dyingmatters.org/sites/default/files/preferred_priorities_for_care.pdf

Universal Principals for Advance Care Planning (ACP)
universal-principles-for-advance-care-planning.pdf

APPENDIX 1 QUALITY OBJECTIVE

Priority 4	**End of Life Care**
Rationale and past performance	End of Life care is a key priority for the Trust. As part of the CQC inspection for Eastern Services in 2016 and Northern Services in 2019 both sites' end life care were rated as good, but also identified areas for improvement which we need to embed into practice. Learning from Structured Judgement Reviews has also demonstrated improvements in our end of life planning. Through this quality objective the Trust will continue to improve the quality of care for patients who are at End of Life • Make the patient the centre of our care and regarded as a partner in the delivery of End of Life care • Ensure staff treat patients and those close to them with sensitivity, courage and compassion
What will we do?	• Promote and support good communication between and within our services • Develop a means of monitoring the quality of care for patients on an End of Life care plan • Make the patient the centre of our care and regarded as a partner in the delivery of End of Life care • Ensure staff treat patients and those close to them with sensitivity, courage and compassion • Work to understand the needs of our patients and families and continue to develop responsive services that are informed by what they have told us • Increase awareness and training on End of Life care for all staff • Develop improved links across our communities to facilitate seamless pathways for patients and carers
Measurable target/s for 2025/26	• Complete the End of Life strategy and launch • Develop & deliver work plan to implement the strategy • monitoring of key delivery milestones through the End of Life Steering Group • Report examples of improved experience for families and staff
How progress will be monitored	• Agreed work plan to implement the strategy and monitoring of milestones through the End of Life Steering Group • Updates to alternate Patient Safety Committee Meetings
Board sponsor	Chief Nursing Officer
Implementation lead	Medical Director and Director of Nursing

Progress to date	Project management support has been identified for the end of life steering group. This will support the development of a SMART work plan to deliver the implementation of the End of Life Strategy.
RAG rating	

APPENDIX 2: COMMUNICATION PLAN

COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the policy	All staff involved in delivering care to a patient or their families
The key changes if a revised policy	This is the first version of the updated policy since the formation of the new Royal Devon University Healthcare NHS Foundation Trust in 2022 and since implementation of EPIC electronic patient records across the organisation; and Version 12 of the regional TEP form. The document has been updated to reflect these local changes, and also regional and national updates.
The key objectives	To ensure all clinical staff working with patients develop a person-centred holistic plan of care to meet the end of life needs, (physical, psychological, spiritual, social and cultural) of the patient and their family To support staff confidence, communication and partnership working through provision of standards of practice and education. To ensure that high quality and compassionate care is delivered in that the privacy, dignity wishes and needs of all patients at the end of life are respected. To enable patients to express their preferred place of dying and for this to be facilitated as far as is reasonably possible. To ensure that the carers are supported, appropriately informed, enabled and empowered throughout the end of life and in the bereavement period of their relative or friend. To ensure there is robust review and targeted education where deficiencies of care are identified.
How new staff will be made aware of the policy and manager action	Local Induction by line manager
Specific Issues to be raised with staff	Staff should be made aware of the policy. Particular attention should be drawn to the responsibilities of Healthcare Professionals in delivering the Five Priorities of Care

Training available to staff	E learning modules available through Learn Plus, Face to Face and experiential learning support available from Specialist Palliative Care Teams, Link Nurse teams for wards, and as part of postgraduate doctor training programmes.
Any other requirements	None
Issues following Equality Impact Assessment (if any)	No negative impacts
Location of hard / electronic copy of the document etc.	The original of this policy, will remain with the author. An electronic copy will be maintained on the Trust Intranet Hub. Archived electronic copies will be stored on the Trust's "archived policies" shared drive, and will be held indefinitely.

APPENDIX 3: EQUALITY IMPACT ASSESSMENT TOOL

Name of document	End of Life Care (Adults) Policy
Care Group/Directorate and service area	Trust-wide
Name, job title and contact details of person completing the assessment	Jess Salvin Nurse Consultant Palliative Care 403570
Date completed:	08/11/2024

The purpose of this tool is to:

- **identify** the equality issues related to a policy, procedure or strategy
- **summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit
- **highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

1. What is the main purpose of this document?

To ensure excellence in the delivery of End of Life Care across the Trust

2. Who does it mainly affect? *(Please insert an "x" as appropriate:)*

Carers ☒ Staff ☒ Patients ☒ Other (please specify)

3. Who might the policy have a 'differential' effect on, considering the "protected characteristics" below? (By *differential* we mean, for example that a policy may have

a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for women than for men)

Protected characteristic	Relevant	Not relevant
Age	☐	☒
Disability	☐	☒
Sex - *including: Transgender, and Pregnancy / Maternity*	☐	☒
Race	☐	☒
Religion / belief	☐	☒
Sexual orientation – *including: Marriage / Civil Partnership*	☐	☒

Apart from those with protected characteristics, which other groups in society might this document be particularly relevant to… (e.g. those affected by homelessness, bariatric patients, end of life patients, those with carers etc.)?

4. Do you think the document meets our human rights obligations? ☒

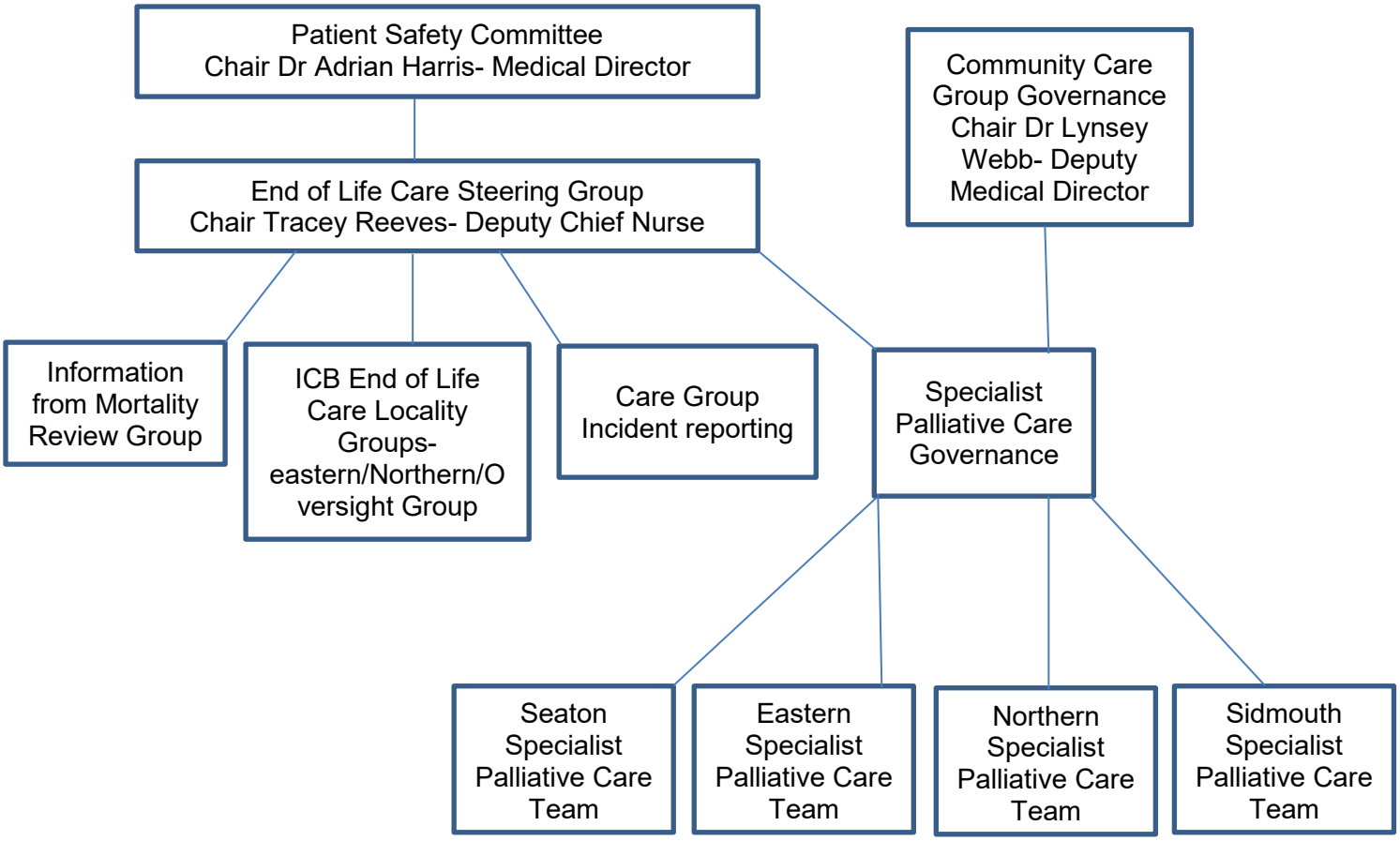
Feel free to expand on any human rights considerations in question 6 below.

A quick guide to human rights:

6. Looking back at questions 3, 4 and 5, can you summarise what has been done during the production of this document and your consultation process to support our equality / human rights / inclusion commitments?

Race and Religion and Belief - Cultural and Spiritual needs often not explored at EOL. Evidence from Trust Audit, NACEL audit data and published reviews shows that families feel that Spiritual need at EOL not being met. We need to explore proactively patient and family needs.

7. If you have noted any 'missed opportunities', or perhaps noted that there remains some concern about a potentially negative impact please note this below and how this will be monitored/addressed.

"Protected characteristic":	
Issue:	
How is this going to be monitored/ addressed in the future:	
Group that will be responsible for ensuring this carried out:	

APPENDIX 4: Specialist Palliative Care and End of Life Care Governance